

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: The patient, at 32 weeks gestation, presents with a medium risk for preeclampsia, indicated by elevated systolic blood pressure (142 mmHg) and proteinuria (dipstick 1). There is also a medium risk for macrosomia, with elevated fasting and postprandial glucose levels and a high pre-pregnancy BMI. An alert to the doctor is recommended for further evaluation of the preeclampsia risk.

Patient Snapshot (Recent Inputs)

Parameter	Value	Notes
Blood Pressure	142/79 mmHg	Hypertensive threshold ≥140/90, severe ≥160/110.
Proteinuria (dipstick)	1	≥1+ supports PE suspicion in context of HTN.
Symptoms	Headache=0, Vision=0, RUQ Pain=0, Swelling=0	Severe features increase escalation priority.
Glucose	Fasting=99 mg/dL 1hr PP=126 mg/dL	Targets typically fasting <95, 1hr <140 (pregnancy goals).
HbA1c	5.6	Higher HbA1c indicates sustained glycemic elevation.
Maternal Context	Age=38 BMI=42.4 GDM=0	Risk modifiers for fetal overgrowth + metabolic risk.
Activity / Sleep	Post-meal walk=29 min Sleep=6.1 hrs	Lifestyle factors influence glycemic response.
Food Log	paneer salad	Used for dietary correlation patterns.

Reason for Alert

Action Requested: ALERT_DOCTOR

Clinical Rationale: ALERT: Patient TEST_MED_MED, G32 weeks, maternal age 38, pre-pregnancy BMI 42.4. Preeclampsia risk tier: medium. Macrosomia risk tier: medium. Key vitals: Systolic BP 142 mmHg, Diastolic BP 79 mmHg, Proteinuria dipstick 1. Fasting glucose 99 mg/dL, 1-hour postprandial glucose 126 mg/dL, HbA1c 5.6%. All severe preeclampsia symptoms (headache, vision changes, upper abdominal pain, swelling) are currently absent. Recommend immediate review of elevated BP and proteinuria in context of medium preeclampsia risk score. Also, evaluate glucose levels and high BMI for macrosomia prevention and GDM management guidance.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

